

VITBION FORTE TABLET

MAL19920011XZ

DESCRIPTION:

TABLET

Colour : Orange
Shape : Round & Biconvex
Coating : Film-coated

CONTENT:

Each tablet contains:

Thiamine HCl..... 100 mg
Pyridoxine HCl 200 mg
Cyanocobalamin 200 µg

PHARMACODYNAMICS:

Thiamine is fundamentally associated with carbohydrate metabolism. By combining with the pyrophosphoric acid in nucleated cells, particularly in the liver, kidneys, and white blood cells, it is converted in the body to its pyrophosphate (cocarboxylase) which acts as a coenzyme in such reactions as the decarboxylation of alpha-keto acids, particularly of pyruvate and alpha-ketoglutarate. In the presence of thiamine deficiency pyruvic and lactic acids accumulate in the tissues. Thiamine pyrophosphate also acts as a coenzyme in the direct oxidative pathway of glucose metabolism.

Pyridoxine is converted to pyridoxal phosphate, which is the coenzyme for a variety of metabolic functions affecting protein, carbohydrate and lipid utilization. Pyridoxine is involved in conversion of tryptophan to niacin or serotonin.

Cyanocobalamin is a member of a group of similar compounds, which influence erythropoiesis. It is used in the treatment of pernicious anaemia and its neurological complication. It is also used in the treatment of other macrocytic anaemias associated with nutritional deficiencies.

PHARMACOKINETICS:

Thiamine is absorbed from the GIT and is widely distributed to most body tissues. It is not stored to any appreciable extent in the body and amounts in excess of the body's requirements are excreted in the urine as unchanged thiamine or as metabolites. About 1 mg of thiamine is metabolised in the body daily.

Pyridoxine is converted to the active form pyridoxal phosphate. Pyridoxal and pyridoxamine are also absorbed and converted. It is excreted in the urine as 4-pyridoxic acid.

Large doses of cyanocobalamin are irregularly absorbed after oral administration is impaired when intrinsic factor is deficient or when gastrointestinal disease is present. It is stored in the liver, undergoes enterohepatic circulation, and crosses the placenta. 50% of a dose is excreted in the urine in 48 hours along with significant amounts in bile.

INDICATIONS:

It is used as a supplement in the treatment of polyneuritis, neuralgias, diabetic neuropathy, drug-induced neuropathy.

RECOMMENDED DOSAGE:

For therapeutic use: 1 tablet 3 times daily.

CONTRAINDICATIONS:

Should not be used in patients hypersensitive to the ingredients.

WARNING AND PRECAUTIONS:

In the literature neuropathies are described under long term intake (6-12 months) of more than 50 mg mean daily dose of Vitamin B₆. Therefore, under long term treatment regular monitoring is recommended.

DRUG INTERACTIONS:

The effect of L-dopa may be reduced when Vitamin B₆ is administered concomitantly.

Pyridoxine-antagonists, e.g. isonized (INH), cycloserin, penicillamin, hydralazine: the efficacy of Vitamin B₆ (Pyridoxine) may be decreased.

Loop diuretics, e.g. Furosemide: In long-term use, the blood level of Thiamine may be reduced.

PREGNANCY AND LACTATION:

No risks have become known associated with the use of VITBION FORTE TABLET during pregnancy at the recommended dosage.

Vitamin B₁, B₆ and B₁₂ are secreted into human breast milk, but risks of overdose for the infant are not known. In individual cases, high doses of Vitamin B₆, i.e. >600 mg daily, may inhibit the production of breast milk.

SIDE EFFECTS/ADVERSE REACTIONS:

Vitamin B groups are water-soluble vitamins and they do not cause any side effects if taken accordingly to the recommended dosage.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdosage rarely occur. Excess Vitamin B will be excreted in the urine.

PACKING/PACK SIZE(S):

Blister pack of 3x10's, 3x12's, 10x10's and 100x10's.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

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